Sutter Health

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June 16, 2025

Dr. Mehmet Oz
Administrator
Centers for Medicare & Medicaid Services

Dr. Thomas Keane, M.D., M.B.A.
Assistant Secretary for Technology Policy & National Coordinator
for Health IT
U.S. Department of Health and Human Services

The Honorable Robert F. Kennedy, Jr.
Secretary
U.S. Department of Health and Human Services

RE: Request for Information: Health Technology Ecosystem (CMS-0042-NC) [RIN 0938-AV68]

Dear Secretary Kennedy, Administrator Oz, and Dr. Keane:

On behalf of Sutter Health, we appreciate the opportunity to respond to the Request for Information issued by the Department of Health and Human Services (HHS), the Centers for Medicare & Medicaid Services (CMS), and the Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology (ASTP/ONC).

Sutter Health is a not-for-profit health care system dedicated to providing comprehensive care throughout California. Committed to community partnerships and innovative, high-quality patient care, Sutter Health is pursuing a bold new plan to reach more people and make excellent health care more connected and accessible. Sutter Health currently serves nearly 3.5 million patients, thanks to a dedicated team of more than 57,000 employees and clinicians and 12,000+ affiliated physicians, with a unified focus on expanding care to serve more patients. Sutter delivers exceptional and affordable care through its hospitals, medical groups, ambulatory surgery centers, urgent care clinics, telehealth, home health, and hospice services.

Sutter is deeply invested in using technology to improve health outcomes, particularly for patients living with chronic conditions. As both a provider organization and a leader in value-based care, Sutter supports CMS, HHS, and ONC's efforts to align regulatory frameworks and payment models with technologies that enhance clinical outcomes, expand equitable access, and advance the broader goals of the Making America Health Again (MAHA) initiative.

We are pleased to highlight how digital tools, when paired with the right payment and policy supports, can transform care for individuals living with hypertension, diabetes, and other chronic diseases. We are also eager to share insights from the implementation of our digital chronic disease management program, Sutter Sync, which combines remote technology with intensive, ongoing care team engagement to keep patients in control of their conditions over time.

1. Digital Health Applications and Provider Adoption (PR-1 to PR-4)

PR-1. Encouraging Provider Use of Digital Health Products

Sutter Health strongly supports efforts to expand access to and adoption of approved digital health tools, particularly for patients managing chronic disease. Traditional primary care models are currently unable to meet the complex and continuous demands of chronic condition management. Providers are forced to practice "an inch deep and a mile wide," and time-constrained visits limit patient engagement and clinical improvement.

Programs like Sutter Sync demonstrate that a virtual-first model, anchored by a dedicated care team and real-time data from biometric devices, can overcome these constraints.[1,2,3] Sync enables longitudinal care management that is always up to date with current clinical guidelines, customized to patient needs, and supported by multilingual teams. It is accessible via cellular service alone and does not require broadband, greatly increasing its accessibility and reach.

To enable a broader provider uptake of similar models, we urge CMS and ONC to:

- Remove reimbursement barriers for Remote Patient Monitoring (RPM) devices;
- Modify reimbursement to support on-going remote patient management, not just periodic monitoring;
- Support standardized integration of device data with EHR platforms like Epic/MyChart; and,
- Incentivize interoperability that consolidates patient-facing apps into unified platforms to drive behavioral change and support varied literacy needs.

PR-1a. Current Obstacles

The fragmented digital landscape and regulatory environment have significantly slowed the pace of innovation in healthcare delivery. RPM devices frequently lack sufficient coverage and are often unaffordable in the absence of direct reimbursement for the device itself. Patients, particularly those managing multiple chronic conditions, are burdened with navigating a multitude of uncoordinated apps, leading to confusion and reduced engagement. On the provider side, there is often inadequate funding to support dedicated care teams tasked with monitoring and responding to incoming data. Providers, already overwhelmed by their existing case load, documentation burden, and concerned with potential liability, are unlikely to adopt or fully utilize new technologies. Technical challenges, such as Bluetooth pairing difficulties and the lack of standardized data formats, further hinder seamless integration with electronic health records. Compounding these issues, fee-for-service reimbursement models that rely on time-

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[1] Milani, R. V., et. al. (2017). Hypertension management in the digital era. *Current opinion in cardiology*, *32*(4), 373–380. https://doi.org/10.1097/HCO.0000000000000405

[2] Milani, R. V., et. al. (2021). New aspects in the management of hypertension in the digital era. *Current opinion in cardiology*, *36*(4), 398–404. https://doi.org/10.1097/HCO.0000000000000870

[3] Liu, M., et. al. (2019) Improving Members Hypertension at Blue Cross Blue Shield of Louisiana: The Effectiveness of Ochsner's Digital Medicine Hypertension (DMH) Program. *Value in Health, 22(S2)*, S135. https://doi.org/10.1016/j.jval.2019.04.528.

based billing fail to account for or reward the true value of achieving and maintaining disease control over time.

In addition to technological fragmentation, financial barriers such as patient copays further inhibit adoption and retention. When patients must pay for each interaction, even those that are brief or primarily preventive, they are more likely to disengage—particularly if their health is periodically stable. This approach punishes success and contradicts the goal of long-term chronic disease management. Eliminating copays for qualifying virtual chronic care would remove an important barrier to patients seeking meaningful care, and could dramatically improve patient retention and outcomes, especially among Medicare beneficiaries.

PR-1b and PR-1c. Information and Responsibilities When Using Digital Tools
Providers have a responsibility to ensure that patients fully understand the intended behavioral outcomes of any digital health program, how their personal health data will be used and protected, and the critical role they play in co-managing their health alongside the care team. For underserved populations, it is especially important that educational content and communication strategies are tailored to accommodate varying levels of health literacy, preferred languages, and access to devices. The Sutter Sync program exemplifies this approach by adjusting care delivery across more than 35 languages and incorporating culturally responsive coaching to meet patients where they are.

PR-2. Obstacles in Physician Workflows
A significant barrier to digital health adoption is the misalignment between innovative care delivery models and the prevailing time-based reimbursement system. The emphasis on time tracking and associated documentation requirements discourages providers from embracing more efficient, technology-enabled approaches. While artificial intelligence (AI) tools have the potential to streamline clinical workflows by flagging patient non-adherence and recommending care pathways, their cost remains unreimbursed. Though larger systems like Sutter Health may be able to overcome some of these challenges through significant upfront and uncompensated investment, existing incentives make this behavior an outlier, and almost unthinkable for smaller practices and community-based providers. To address these challenges, CMS should consider shifting reimbursement toward clinical outcomes rather than the duration of interactions, investing in the infrastructure necessary to support dedicated virtual care teams, and promoting access to affordable AI tools that can enhance care coordination and decision-making.

PR-3. Importance of EHR Data Accessibility
Effective chronic disease management in real-world settings depends on clinicians and care teams having real-time access to comprehensive health data. Providers must be able to view biometric and behavioral data longitudinally to understand trends over time, track sampling rates and key control metrics such as blood pressure, LDL cholesterol, and A1c levels, and receive timely alerts that enable proactive intervention before a patient's condition worsens.

PR-4. Enabling Third-Party App Access to Administrative Workflows
Similarly, enabling third-party applications to support administrative workflows requires more than just access to patient data. Integration must extend to core operational functions such as scheduling, referral tracking, and team-based communication. To facilitate this, CMS and ONC

should strengthen support for standardized APIs that ensure cross-platform functionality, promote automated tasking within electronic health records to support coordinated chronic care management, and enhance interoperability between care navigation platforms and payer administrative systems. These steps are essential for building a cohesive digital health ecosystem capable of delivering high-quality, coordinated care at scale.

2. Digital Health in Value-Based Care Models (VB-1 to VB-4)

VB-1. Incentives to Drive ACO/MSSP Adoption

A key challenge in value-based care is sustaining long-term chronic disease control. Based on our experience in managing these populations, a sizeable percentage of patients who initially achieve disease stability will fall out of control within a year. Accountable Care Organizations (ACOs) and participants in the Medicare Shared Savings Program (MSSP) must therefore be equipped with digital platforms that enable continuous engagement. Unfortunately, current fee-for-service (FFS) time-based billing codes do not support the low-intensity, ongoing check-ins that help prevent costly acute events. To address this gap, Sutter Health recommends that CMS adopt a per-member-per-month (PMPM) payment model tied directly to chronic condition control rates. Additional policy enhancements could include retrospective, performance-based bonuses for achieving population-level health benchmarks and the elimination of patient cost-sharing for digital chronic care services, modeled after ACO REACH flexibilities.

Patient co-pays present a substantial barrier to the success of digital chronic care programs. Even patients who are responding well to care may question the value of continued participation when they are required to pay out-of-pocket for brief, low-touch interactions. For individuals on fixed incomes, especially those with Medicare, copays can deter ongoing engagement, undermining preventive care efforts and increasing the risk of costly downstream events. CMS should consider policy reforms that remove or minimize patient cost-sharing for virtual chronic care management, aligning financial incentives with sustained patient participation and long-term condition control.

VB-2. Integrating Technologies into APMs

To fully realize the potential of digital health technologies within alternative payment models (APMs), integration must go beyond data collection. Effective APMs should incorporate artificial intelligence tools that stratify patient risk and flag non-adherence, as well as behavioral science-based coaching methods that promote health literacy and sustained engagement. Seamless alignment of digital monitoring tools with human-led care management is essential. Sutter Sync's approach illustrates this distinction clearly as its strength lies in active, guided, outcome-driven management rather than passive monitoring.

VB-3. Essential Health IT Capabilities

To support scalable, effective virtual chronic care management, CMS and ONC should prioritize key health IT capabilities. These include: embedding tools within electronic health records (EHRs) that allow longitudinal tracking of key control measures; enabling real-time transmission alerts to care teams for timely intervention; using sampling logic to monitor program reach and clinical effectiveness; and providing advanced dashboard analytics that help ACOs monitor

population health trends and flag at-risk patients. These capabilities are foundational to any care model seeking to improve chronic condition outcomes at scale.

VB-4. Essential Data Types

Chronic disease management requires reliable access to a wide range of data types. This includes biometric data from devices, claims and encounter data to evaluate cost and utilization patterns, and social determinants of health (SDOH) data that inform targeted outreach and individualized intervention. Future alternative payment models should enable persistent access to these integrated data sources, not just at the point of care, but throughout the entire continuum of disease management. This longitudinal view is critical to identifying trends, managing risk, and improving health outcomes over time.

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We again appreciate your consideration of our comments. Should you have questions, please do not hesitate to contact me by phone at 916-718-2085 or email at Jonathan.Williams@sutterhealth.org.

Sincerely,

Richard Milani, MD
Chief Clinical Innovation Officer
Sutter Health